

Al Noor General Hospital

Infection Control and Occupational Health Policy

Version 3.0 | Effective January 2026 | Sample document for testing

1. Purpose and Infection Control Structure

1.1 Purpose. This policy protects patients, visitors and staff of Al Noor General Hospital from healthcare-associated infections. It applies to every person who works in or visits the hospital.

1.2 Infection Control Unit. The Infection Control Unit (ICU, not to be confused with the Intensive Care Unit) is led by the Infection Control Officer and is reachable on extension 240. The unit provides advice 24 hours a day, trains staff, carries out audits and investigates suspected outbreaks.

1.3 Infection Control Committee. The committee meets every month. It includes the Medical Director, the Director of Nursing, the Infection Control Officer, a microbiologist, the head of housekeeping and a pharmacy representative.

1.4 Standard precautions. Standard precautions apply to all patients at all times, whether or not infection is suspected. They include hand hygiene, use of personal protective equipment, safe handling of sharps, safe waste disposal, cleaning of the environment and respiratory etiquette.

1.5 Staff responsibility. Every staff member is personally responsible for following this policy. Staff who see unsafe practice must speak up politely and report it to the department head or the Infection Control Unit.

1.6 Related documents. Rules about visitors, discharge and medical records are set in the Patient Care and Safety Policy Manual. Staff leave and incident reporting rules are set in the Staff Handbook. This policy repeats only what staff need for infection prevention.

2. Hand Hygiene

2.1 Five Moments. Staff must clean their hands at the 5 Moments of Hand Hygiene: before touching a patient, before a clean or aseptic procedure, after body fluid exposure, after touching a patient, and after touching patient surroundings.

2.2 Alcohol-based hand rub. Alcohol-based hand rub is available at every bed and at every ward entrance. Hands are rubbed for at least 20 seconds, covering all surfaces, until dry.

2.3 Soap and water. Staff must wash their hands with soap and water for at least 40 seconds when hands are visibly dirty or soiled with blood or body fluids, after using the toilet, and after caring for a patient with diarrhoea or a spore-forming infection.

2.4 Hand care. Staff keep fingernails short and clean. Artificial nails, nail polish that is chipped, and rings with stones are not allowed in clinical areas. Cuts and abrasions on the hands must be covered with a waterproof dressing.

2.5 Gloves. Gloves do not replace hand hygiene. Hands must be cleaned before putting on gloves and after taking them off. Gloves are changed between patients and between tasks on the same patient.

2.6 Audit. The Infection Control Unit observes hand hygiene practice every month in each ward and shares the compliance rate with the ward team. Wards with low compliance receive extra coaching.

3. Personal Protective Equipment (PPE)

3.1 Risk assessment. Before every task, staff assess the risk of contact with blood, body fluids, secretions or contaminated surfaces, and choose the PPE that matches the risk.

3.2 Types of PPE. PPE includes disposable gloves, surgical masks, N95 respirators, gowns or aprons, and eye protection such as goggles or face shields.

3.3 Changing PPE. PPE such as gloves, masks and gowns must be changed between patients. PPE is removed before leaving the patient's room, except for the respirator, which is removed outside the room after the door is closed.

3.4 Putting on and taking off. Staff put on PPE in this order: gown, mask or respirator, eye protection, gloves. They take off PPE in this order: gloves, eye protection, gown, mask or respirator, followed by hand hygiene.

3.5 Fit testing. Staff who may wear an N95 respirator are fit tested when they join and then every year. A respirator must be worn only by a staff member who has passed fit testing for that model.

3.6 Supplies. Each ward keeps PPE stock at the entrance of isolation rooms. The ward manager informs the stores department when stock falls below the agreed minimum level.

3.7 Reuse. Single-use PPE is never reused or washed. Reusable items such as goggles are cleaned and disinfected after each use.

4. Isolation Precautions

4.1 Types of isolation. There are three types of transmission-based precautions: contact precautions, droplet precautions and airborne precautions. They are used in addition to standard precautions.

4.2 Contact precautions. Used for infections spread by touch, such as resistant bacteria and skin infections. Staff wear gloves and a gown on entering the room. Equipment is kept in the room for the use of that patient only.

4.3 Droplet precautions. Used for infections spread by large droplets, such as influenza. Staff wear a surgical mask when within 1 metre of the patient. The patient wears a mask when leaving the room.

4.4 Airborne precautions. Used for infections such as tuberculosis, measles and chickenpox. The patient is placed in a negative-pressure room with the door closed. Staff wear a fit-tested N95 respirator before entering.

4.5 Signage. A coloured isolation sign is placed on the door of every isolation room. The sign shows which precautions apply, but never shows the diagnosis or the patient's name.

4.6 Patient movement. Patients in isolation leave their room only for essential tests or treatment. The receiving department is told in advance so that precautions can be ready.

4.7 Ending isolation. Isolation is stopped only when the Infection Control Unit or the treating doctor confirms it. The room then receives terminal cleaning before the next patient is admitted.

5. Sharps Safety and Needle-stick Injuries

5.1 Safe handling. Staff who use needles or blades must dispose of them in a sharps container immediately after use, at the point of use. Needles must never be recapped by hand, bent or broken.

5.2 Sharps containers. Sharps containers must be puncture-resistant and are fixed at a safe height. They are sealed and replaced when they are three-quarters full and never pressed down to make room.

5.3 Safety devices. The hospital supplies safety-engineered needles and syringes. Staff must use them whenever they are available.

5.4 First aid after injury. After a needle-stick or sharps injury, or a splash of blood or body fluid to the eyes, mouth or broken skin, the person must immediately wash the area with soap and running water. Eyes are rinsed with clean water or saline. The wound must not be squeezed or scrubbed.

5.5 Reporting. Every needle-stick or sharps injury must be reported to the Infection Control Unit within 1 hour of the injury, by calling extension 240. The injured person must then go to Occupational Health for assessment.

5.6 Follow-up. Occupational Health tests the injured staff member and, when possible, the source patient, and decides whether post-exposure treatment is needed. Follow-up blood tests are done as advised by the doctor. The injury must also be entered in the incident reporting system within 12 hours.

5.7 Confidentiality. The results of testing after an injury are kept confidential and shared only with the staff member and the Occupational Health doctor.

6. Clinical Waste Management

6.1 Segregation at source. Waste is separated at the place where it is produced. Staff must never put clinical waste into general waste bins.

6.2 Colour codes. Yellow bags are used for infectious clinical waste, such as dressings, used gloves and masks soiled with body fluids. Black bags are used for general non-infectious waste. Sharps go to sharps containers only. Pharmaceutical waste goes to the labelled pharmacy waste containers.

6.3 Handling. Bags are closed when they are three-quarters full, labelled with the ward name and date, and moved to the ward dirty utility room. Bags must not be carried against the body or dragged along the floor.

6.4 Transport and storage. Housekeeping staff move waste in covered trolleys using designated routes and lifts at times that avoid meals and visiting hours. The central waste store is locked and cleaned every day.

6.5 Spills. Blood and body fluid spills are covered with absorbent material, cleaned with a chlorine-based disinfectant and then cleaned again with detergent. The person cleaning wears gloves and an apron.

6.6 Linen. Used linen is placed in a laundry bag at the bedside without shaking it. Infected linen is placed in a water-soluble bag and then in a red laundry bag.

6.7 Training. All housekeeping and nursing staff are trained in waste segregation during induction and every year after.

7. Cleaning, Disinfection and Sterilization

7.1 Routine cleaning. Patient rooms are cleaned every day. High-touch surfaces, such as bed rails, call buttons, door handles, light switches and over-bed tables, are cleaned at least twice a day.

7.2 Terminal cleaning. After every discharge and after isolation ends, the room receives terminal cleaning. All surfaces, the mattress cover, the curtains and the bathroom are cleaned and disinfected before the next patient arrives.

7.3 Equipment. Shared equipment, such as thermometers, stethoscopes, blood pressure cuffs and glucose meters, is cleaned and disinfected between patients, following the manufacturer's instructions.

7.4 Disinfectants. Staff use only hospital-approved disinfectants at the correct dilution and contact time. Products must not be mixed. Safety data sheets are kept in every housekeeping store.

7.5 Central Sterile Supply Department. Reusable surgical instruments are processed in the Central Sterile Supply Department (CSSD). The flow runs one way from the dirty area to the clean area to the sterile store.

7.6 Sterilization. Instruments are sterilized in a steam autoclave at 134 degrees Celsius. Every load is checked with chemical and biological indicators, and the results are recorded and kept. A load that fails any indicator is recalled and reprocessed.

7.7 Sterile packs. Sterile packs are checked for damage, wetness and expiry before use. A damaged or wet pack is not used and is returned to CSSD.

8. Outbreak Management and Reporting

8.1 Definition. An outbreak is suspected when two or more patients or staff on the same ward develop the same infection within a short time and a link is likely.

8.2 Immediate action. Any staff member who suspects an outbreak must telephone the Infection Control Unit on extension 240 immediately and inform the nurse in charge. The incident form is then completed within 12 hours.

8.3 Outbreak team. The Infection Control Officer convenes the outbreak team, which includes the treating doctors, the ward manager, a microbiologist and the Quality Department. The team confirms the diagnosis, identifies how the infection is spreading and decides on control measures.

8.4 Control measures. Measures may include isolating or grouping affected patients, closing a ward to new admissions, extra cleaning, screening of contacts, and restricting visitors. Any change to visiting is communicated by the Infection Control Unit and the ward manager.

8.5 Communication. Affected patients and their families are informed honestly by the treating doctor. The Medical Director informs the health authority when this is required by law.

8.6 Ending the outbreak. The outbreak is declared over only after no new cases have appeared for the period set by the outbreak team, and the final cleaning is complete.

8.7 Learning. A written report with the cause, actions taken and lessons learned is presented at the next Infection Control Committee meeting.

9. Staff Vaccination and Occupational Health

9.1 Pre-employment screening. All new staff complete a health screening with Occupational Health before they start work. Records of previous vaccination and immunity are collected.

9.2 Hepatitis B. Staff who may be exposed to blood or body fluids must complete the hepatitis B vaccination course, and their immunity must be confirmed, before they have contact with patients.

9.3 Influenza. The hospital offers free seasonal influenza vaccination to all staff every year. Clinical staff are required to be vaccinated or to wear a mask in patient areas during the influenza season.

9.4 Other vaccines. Measles, mumps, rubella, varicella (chickenpox) and tetanus immunity are checked, and vaccines are offered free to staff who are not immune.

9.5 Working when ill. Staff with fever, vomiting, diarrhoea, a new cough or a skin infection must not work in patient areas. They must tell their department head and contact Occupational Health before returning.

9.6 Return to work. Staff with diarrhoea or vomiting stay away from work until they have had no symptoms for 48 hours. Other illnesses are assessed individually by Occupational Health.

9.7 Confidentiality. Occupational Health records are confidential and are kept separate from the staff HR file. Managers are told only whether a person is fit to work and any restriction that applies.

10. Visitors, Audit and Continuous Improvement

10.1 Visitors and infection prevention. The hospital's visiting rules also protect patients from infection. Visiting hours are 4:00 PM to 8:00 PM, with a maximum of 2 visitors per patient at a time. Children under 12 years are not allowed in general wards without written approval from the ward nurse in charge.

10.2 Sick visitors. Visitors with fever, cough, diarrhoea or any other infectious symptom must not enter patient areas. Staff may ask visitors to leave and to come back when they are well.

10.3 Visitor hand hygiene. Visitors must clean their hands with alcohol-based hand rub when entering and leaving the ward, and before and after touching the patient. Visitors must not sit on patient beds or use patient toilets.

10.4 Isolation rooms. Visitors to isolation rooms must report to the nurse first. The nurse explains the precautions and shows how to put on and take off any PPE. Visitors to patients in airborne isolation are limited to those approved by the nurse in charge.

10.5 Surveillance. The Infection Control Unit tracks infections such as catheter-related bloodstream infections, urinary catheter infections, surgical site infections and ventilator-associated pneumonia, and reports the rates to the committee every month.

10.6 Audit and feedback. Wards are audited for hand hygiene, PPE use, cleaning, sharps and waste. Results are shared with the ward manager, and an action plan is agreed for every audit that falls below target.

10.7 Education. Every staff member receives infection control training at induction and then every year. Training records are kept by the Learning and Development team.